

ISSUE BRIEF

Supporting LGBTQ+ Older Adults' Basic Needs

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INTRODUCTION

LGBTQ+ older adults face significant challenges. Recently, the rise of targeted threats against the LGBTQ+ community,¹ including the weaponization of policies to erase LGBTQ+ people, significant cuts and restrictions to safety net programs like Medicaid,² and the lack of enforcement of anti-discrimination protections,³ harm LGBTQ+ older adults. Despite their indisputable existence throughout history and important contributions in our communities, LGBTQ+ older adults continue to be systemically targeted in unconscionable ways by policymakers. This issue brief highlights improvements to programs and services that would enable LGBTQ+ older adults to age with dignity in the United States.

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It is not a color of the rainbow, but [grey can be gay](#). You just have to learn to replace casual intimacies with casual wisdom.

—MARTIN BOYCE, SAGE PARTICIPANT

Note: In this resource, we use the acronym LGBTQ+ to describe the lesbian, gay, bisexual, transgender, queer, and gender expansive community. The use of this broad-encompassing acronym reflects SAGE and Justice in Aging’s commitment to uplifting the diverse perspectives in the LGBTQ+ older adult community. Occasionally, however, this issue brief uses the acronym LGBT or LGB when that is the language used in the cited source.

DISCRIMINATION AND POVERTY AMONG LGBTQ+ OLDER ADULTS

There are an estimated 3.6 million older LGBTQ+ people in the United States, and by the year 2030, there will be seven million LGBTQ+ people over the age of 50.⁴ Despite their growing numbers, LGBTQ+ older adults face significant inequities and discrimination in terms of access to health care, housing, and economic security. Health inequities stem from stigmatization in medical research, lack of social supports and resources, and medical mistreatment (such as lack of gender affirming care), which ultimately lead to a greater likelihood of depression, dementia, and chronic health conditions.⁵ The seeds of economic inequity are sowed at a younger age. For example, workplace discrimination against LGBTQ+ people earlier in their lives—and in particular transgender people—results in not being hired for certain jobs, not being promoted, being fired, or not being paid equitably due to one's sexual orientation or gender identity. The compounding effect of a lifetime of discrimination leads many LGBTQ+ people to age into poverty. These impacts are exaggerated in specific situations, such as the COVID-19 pandemic (e.g., more LGBT older adults reported living in poverty and experiencing food insecurity than their non-LGBT counterparts)⁶ and incarceration (85% of incarcerated LGBTQ people have been put in solitary confinement).⁷

For LGBTQ+ older adults who belong to other historically marginalized communities, systemic inequities can be compounded or unique. LGBTQ+ people of color experience heightened rates of discrimination and hate crimes.⁸ For transgender people of color, these experiences are unfortunately too common—with 68% reporting negative or discriminatory treatment from health care providers and 28% reporting that they had a doctor

refuse to see them based on their gender identity. LGBTQ+ people of color are also more likely than their white counterparts to experience workplace discrimination—including harassment, negative comments, not being considered for promotions—ultimately leading to economic inequity that compounds over one's lifetime.⁹ Approximately 30% of LGBTQ+ people report having a disability, which can increase discrimination in health care and employment settings and increase the risk of domestic violence.¹⁰ As a result of these harmful discriminatory experiences, many LGBTQ+ older adults who have other marginalized identities may avoid health care or other services or choose not to out themselves to their employer, providers, or caregivers.¹¹

The combination of aging and trauma brings distinctive challenges for older adults as they grapple with the cumulative effects of past and present traumatic experiences. Many LGBTQ+ older adults lived through state-sanctioned discrimination and health-related trauma perpetuated by institutions.¹² Legalized discrimination against the LGBTQ+ community reinforces institutional oppression,¹³ and the most recent round of policy attacks against LGBTQ+ communities through executive orders and other administrative actions increases inequities for LGBTQ+ older adults. Discrimination that highlights the lack of legal protections for LGBTQ+ individuals in areas like housing, health care, education, and employment, financially penalize the community throughout their lives.¹⁴

Given the diversity within the LGBTQ+ community and the challenges LGBTQ+ older adults experience, below we detail ways that health care, economic, and other social support programs can meet the needs of LGBTQ+ older adults.

SAL'S STORY: A LIFETIME LIVED FROM 1952 TO 2025, FRAMED THROUGH THE LENS OF SOMEONE WHO IS 85 YEARS OLD IN 2025.

Over the course of 85 years, Sal has witnessed profound shifts in LGBTQ+ rights and visibility. At age 12, during the early 1950s, the McCarthy hearings targeted “homosexuals,” and the American Psychiatric Association (APA) listed homosexuality as a mental disorder. By age 29, Sal saw the Stonewall Riots in New York City, a pivotal moment that ignited the modern LGBTQ+ rights movement. At 33, the APA removed homosexuality from its list of mental disorders, marking a significant step toward destigmatization. The AIDS crisis began when they were 43, bringing fear and prejudice but also galvanizing activism. At 54, the “Don’t Ask, Don’t Tell” policy was enacted, barring openly LGBTQ+ individuals from military service. At age 63, the Supreme Court struck down laws criminalizing same-sex sexual conduct in *Lawrence v. Texas*. By 75, Sal witnessed the legalization of same-sex marriage nationwide through *Obergefell v. Hodges*, followed by the designation of the Stonewall Monument as a U.S. National Monument at age 76. At 80, *Bostock v. Clayton County* extended federal employment protections to LGBTQ+ workers. At 82, a transgender woman won a landmark settlement in *King v. Sunrise Assisted Living*.¹⁵ At 84, gender identity and sexual orientation were added to the definition of “Greatest Social Need” under the Older Americans Act. Now, at 85, Sal is living through a time marked by a surge in attacks via executive orders and legal challenges aimed at dismantling LGBTQ+ rights, with organizations like SAGE and Justice in Aging fighting back through litigation and administrative advocacy. This timeline reflects both the resilience and the ongoing struggle for equality across generations.

LGBTQ+ GENERATION TIMELINE¹⁶

LGBTQ+ generation name	Birth year range	Age range (as of Jan. 2024)	Cohort experiences	Mainstream generation name
Invisible Generation	1934 and earlier	90 years+	Sexual and gender identities were “invisible”; WWII, Great Depression; increasing gender fluidity as women entered labor force during WWII; medical and legal institutions defined homosexuality as an “inclination”	Greatest Generation (born 1901–1924)
Silenced Generation	1935–1949	75–89 years	Lavender Scare; medical and legal institutions defined homosexuality as a “perversion”; Kinsey scale introduced	Silent Generation (born 1925–1945)
Pride Generation	1950–1964	60–74 years	Civil rights era; reproductive rights; rise of collective activism; Stonewall; creation of rainbow flag	Baby Boomer Generation (born 1946–1964)
Caregiver Generation	1965–1979	45–59 years	AIDS epidemic; Defense of Marriage Act (DOMA); Don’t Ask, Don’t Tell	Generation X
Equality Generation	1980–1996	26–44 years	Marriage equality; repeal of Don’t Ask, Don’t Tell and DOMA; more public figures coming out	Millennials
Ze Generation	1997–2013	11–25 years	Gender diversity; fluidity; rise of discriminatory laws based on gender expression and identity (trans exclusions)	Generation Z

USING DATA TO ADVANCE EQUITY

Data collection is necessary to better understand the unmet needs of LGBTQ+ older adults and their experiences with federal and state benefit programs. While the programs discussed in this issue brief gather and analyze information about individuals, they rarely collect data on sexual orientation and gender identity (SOGI). Moreover, data that is collected is not always analyzed intersectionally—for example by age and gender identity. Equitable data collection also requires survey participants to feel comfortable in disclosing SOGI information to agencies, which requires policies that make LGBTQ+ people feel safe and included. Some examples include ensuring individuals know their data will not be used against them or result in other harmful consequences, deploying culturally competent direct-services staff, using imagery that is representative of the LGBTQ+ community, and having clearly articulated and enforced anti-discrimination and privacy protections that are easy to navigate.¹⁷

Until recently, the federal government was making strides toward data collection that more accurately captured the experiences of LGBTQ+ older adults following decades of advocacy. However, the administration's attacks against LGBTQ+ people have reversed this progress. Not only have most SOGI data collection efforts been halted and some even scrubbed,¹⁸ but trainings and policies that enable culturally competent data collection are also being targeted as "illegal DEI."¹⁹ Recent data abuses by the federal government (including sharing Medicaid beneficiaries' personally identifiable information with immigration agents for the purposes of deportation action) further breeds marginalized communities' mistrust that personal data shared with the federal government may ultimately be used against them.

HEALTH PROGRAMS

Medicare

Medicare is a federal health insurance program available to people age 65 and older or people with certain disabilities. Between 595,00 and 790,000 LGBTQ+ older adults are estimated to be enrolled in Medicare in 2021, with 69% of LGBT older adults nationwide relying on Medicare as their primary source of health insurance.²⁰ Medicare coverage includes hospital care (Part A) and doctor's visits (Part B) and can include prescription drug coverage (Part D).²¹

In addition to the age or disability requirements, all older adults, including LGBTQ+ older adults, must have an appropriate work history to receive Medicare Part A (i.e., hospital insurance) premium-free, which is roughly the equivalent of 10 years of taxable employment. Given the barriers to LGBTQ+ people gaining and maintaining employment—such as discrimination in hiring practices and homophobic or transphobic harassment that may lead them to leave a job²²—this requirement can be a barrier to LGBTQ+ older adults accessing Medicare. Moreover, many LGBT people are more likely to have unpaid caregiving responsibilities than non-LGBT counterparts, an essential role that does not count toward the requisite work history.²³

While a person who lacks the requisite work history can still access Medicare if their spouse—living or deceased—meets the requirements, this option is inequitable as it disadvantages LGBTQ+ older adults for whom marriage was not an option until relatively recently. A person without qualifying quarters can enroll in Medicare but will be required to pay a premium to access Part A hospital insurance, which can amount to up to over \$500 per month.²⁴ Paying the premium out-of-pocket is difficult for LGBTQ+ older adults who experience economic inequities, and thus they may rely on Medicaid to pay the premium.

Medicaid

Medicaid is a state-administered health insurance program available to qualifying people with limited income and assets, the thresholds of which vary by state. It is a lifeline for many people, including LGBTQ+ older adults. LGBT (13%) adults are twice as likely as non-LGBT (7%) adults to have Medicaid as their primary source of health insurance, and an estimated 1.8 million LGBTQ+ adults are enrolled in Medicaid.²⁵ Medicaid is particularly beneficial for LGBTQ+ older adults who are not yet Medicare age (i.e., in the 50-64 range) and do not have other sources of health care coverage.

Medicaid is also an important benefit for LGBTQ+ older adults who do have Medicare. As explained previously, for those without adequate work histories to access premium free Part A hospital insurance, Medicaid will pay the cost of the premium.²⁶ Moreover, it will also help to pay for other coinsurance costs—such as co-pays at the doctor and drug costs—thus freeing up sometimes limited income for LGBTQ+ older adults to pay for rent, groceries, and other necessities.²⁷ Finally, Medicaid allows LGBTQ+ older adults to access important health care services that are not available through Medicare, all of which allow LGBTQ+ older adults to age with dignity in the community. These include dental, vision, hearing, and long-term care, including home- and community-based services (HCBS).

LGBTQ+ older adults often face barriers remaining at home as they age as they may not have traditional sources of unpaid care from family. Compared to non-LGBTQ+ older adults, LGBTQ+ older adults are twice as likely to be single and four times less likely to have children.²⁸ Unpaid caregiving support from friends and neighbors is never guaranteed and is especially challenging for those living in rural areas. When it is available, it lacks the legal protections enforced on providers (such

as anti-discrimination laws) and its long-term sustainability is precarious, especially as those providing unpaid care may themselves be older adults.²⁹ The alternative of institutional care (i.e., nursing facilities) is not only undesirable for many older adults, but can also present threats of cultural unresponsiveness, discrimination, exclusion, isolation, and denial of essential care for LGBTQ+ older adults. Indeed, many LGBTQ+ older adults have reported discrimination and having to hide their identity in nursing facilities.³⁰ Medicaid-funded HCBS empowers millions of older adults—including LGBTQ+ older adults—to age in their communities with the help of paid care aides, home modifications, and other supports that enable them to live at home. These services are governed by anti-discrimination rules detailed later. Furthermore, protections for LGBTQ+ older adults and those with HIV may be further reiterated at the state level, such as through implementing the LGBTQ+ and HIV Long-Term Care Bill of Rights—a policy initiative that codifies protections in the context of nursing facilities, assisted living facilities, and home- and community-based services programs.³¹

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Living in a rural area is hard as you age. Health care is 1.5 hours away (if you can even find a doctor), there is no public transportation at all, no senior housing nearby. The few age-related services (i.e., senior centers) are not gay friendly. There are no funds to support a gay program for the elderly.

—ANONYMOUS, LGBTQ+ OLDER ADULT

HIV CARE AND GENDER AFFIRMING CARE

Medicare and Medicaid are essential for LGBTQ+ older adults requiring HIV care and gender affirming care. Older adults are disproportionately impacted by HIV worldwide.³² In the U.S., gay and bisexual men of color (especially Black and Latino men who have sex with men) and transgender people experience higher rates of HIV than other populations.³³ Fifty-four percent of all people living with HIV in the United States are aged 50 or older.³⁴

Medicaid is the nation's largest source of coverage for people with HIV, and the majority of Medicaid beneficiaries with HIV are dually enrolled in Medicare.³⁵ Medicaid covers a broad range of services that are critical for people with HIV, ranging from doctor's appointments, hospital visits, laboratory services, prescription drugs, and HCBS. LGBTQ+ older adults who are younger than 65 and have HIV are also eligible for Medicare,³⁶ which is the second largest source of federal funding for HIV care in the U.S..³⁷ Another crucial source of medical care and support is the Ryan White HIV/AIDS Program, which, like Medicare and Medicaid, is administered by the federal Department Health and Human Services (HHS) and Health Resources and Services Administration (HRSA).

Medicaid and Medicare also play an important role in making gender affirming care accessible to LGBTQ+ older adults. Gender affirming care includes health care services and inclusive approaches that support an individual's gender identity, including hormone therapies, psychiatric services, and primary care.³⁸ A 2022 study estimated that 60% of transgender Medicaid beneficiaries have affirmative access to coverage for gender-affirming care under express policies in state law, which facilitates an improved quality of life for transgender and gender expansive older adults.³⁹ However, a number of states are passing bans on gender affirming care for youth, which may have implications for older adults. To date, Medicare has covered gender affirming services—such as gender reassignment surgery—on an individual basis when deemed medically necessary.⁴⁰



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When people are in good, competent care, they feel empowered to access their healthcare. There is certainly some **trepidation with my LGBTQ+ clients when it comes time to find a new doctor or specialist.**

People think ‘I have to go through this all over again, I have to tell them everything about myself, and you never know how it is going to be taken.’ There is a lot of fear around Medicare/Medicaid this year. They often ask, **‘Will I still get my HIV medication? How am I going to pay my bills? What else is going to be taken away?’**

This is not just LGBTQ+ people, I do see this across the board, but **I see it especially with LGBTQ+ elders because we have special circumstances.** It will be interesting to revisit this conversation in January after the open enrollment period to see what has changed and how that has impacted people in their everyday lives. Luckily, most of my clients, even in conservative states like Florida, are in more urban areas, which offers a certain level of access.

—STEPHANIE SMITH, LGBTQ+ ELDER AND INDEPENDENT LICENSED MEDICARE AGENT, ST. PETERSBURG, FL

ANTI-DISCRIMINATION PROTECTIONS IN HEALTH CARE

Section 1557 of the Affordable Care Act prohibits discrimination in health care programs and services that receive federal funds—i.e., virtually all health care providers, settings, and services. A 2024 rule clarified that its protections extend to discrimination on the basis of sex, gender identity, sexual orientation, and sex characteristics and specifically banned the denial of gender affirming care, but these regulations have been challenged in court.⁴¹ As of May 2025, the Department of Health and Human Services (HHS) has rescinded previous sub-regulatory guidance that articulated the agency’s interpretation of Section 1557 to prohibit discrimination on the basis of sexual orientation and gender identity and related enforcement.⁴² Although HHS’s interpretation and enforcement has shifted, Section 1557’s statutory language remains unchanged and continues to prohibit discrimination in health care on the basis of sex, race, ethnicity, age, and disability (including HIV).⁴³

Section 504 of the Rehabilitation Act prohibits discrimination on the basis of disability. HHS has previously reiterated its commitment to protecting LGBTQ+ people in rulemaking stating that LGBTQ+ individuals with gender dysphoria or HIV are protected by these laws in programs that receive federal funds, including health care settings.⁴⁴ Right-wing states challenged the regulations in 2024, and in 2025 the federal government indicated it will not be enforcing nondiscrimination protections for people with gender dysphoria under Section 504.⁴⁵

ECONOMIC SUPPORT PROGRAMS

Social Security Benefits

People with requisite work histories—roughly ten years of earning taxable income—can access Social Security Retirement or Social Security Disability Insurance. The workplace inequities that LGBTQ+ people experience can limit their ability to access or remain in jobs that contribute to Social Security-earning work history, and ultimately negatively impact their ability to access these benefits in older age. Much like Medicare, older adults who do not have the requisite work history might be able to access Social Security benefits if their spouse qualifies. Accordingly, for decades, same-sex couples could not access spousal benefits at all. Although this restriction was reversed in 2015 following the United States Supreme Court’s decision in *Obergefell v. Hodges*, many LGBTQ+ older adults may face barriers in getting Social Security spousal benefits due to stigma, lack of information, and increasing attacks against the LGBTQ+ community that impact couples’ decision to formally marry.⁴⁶ In fact, a 2021 study showed that nearly 50% of LGBTQ+ people 45 and older are single, and this disparity is even greater for gay men and transgender and nonbinary people.⁴⁷ Even post-*Obergefell*, advocates had to secure survivor’s benefits for same-sex partners through litigation in the courts.⁴⁸

In a reversal from previous policy, the Social Security Administration will no longer process changes to gender markers, though name changes remain permissible. Individuals who previously changed their gender marker are not impacted by the policy reversal. The inability to change one’s gender marker may present complications for transgender older adults who possess inconsistent documents.

Supplemental Security Income

LGBTQ+ older adults aged 65 and older who have very limited income and resources can access Supplemental Security Income (SSI).⁴⁹ SSI is a lifeline for individuals who have little to no other income as it is available to individuals earning \$967 or less per month, including through sources like pensions, Social Security benefits, in-kind donations, and other cash assistance programs.⁵⁰ SSI benefits are crucial for LGBTQ+ older adults who have experienced a lifetime of economic inequities. For example, 25% of transgender and nonbinary older adults have reported that they earn less than \$25,000 annually.⁵¹ Relatedly 28% of LGBT adults have no emergency funds.⁵²

HOUSING, FOOD, AND OTHER SUPPORT PROGRAMS

Affordable Housing and Homelessness Assistance

Housing concerns impact many older adults, but for the more than 2.7 million LGBTQ+ adults age 50 or older,⁵³ there are added challenges that have built up over a lifetime. Although lack of long-term data makes it difficult to adequately document the challenges faced, we know that fear of discrimination and economic inequities have contributed to housing inequities. For example, years of employment discrimination can result in lower retirement savings and Social Security income. In health care, challenges accessing medical care and aging-specific services can increase health risks, and, with fewer connections to families of origin, LGBTQ+ older adults may be at increased risk for social isolation. Without savings, confronted with higher-than-average medical bills and lacking younger family members to fall back on for support, the risk of housing instability increases for LGBTQ+ older adults. Affordable and inclusive

housing programs—such as Housing Opportunities for Persons with AIDS Program (HOPWA) and federal rental assistance programs—are necessary for LGBTQ+ older adults and others to age in their communities.⁵⁴

Compared with the 70% of non-LGBTQI+ adults who own their homes, only 54% of all LGBTQI+ adults are homeowners⁵⁵ and one study found that 12% of LGBT adults experience housing hardships.⁵⁶ The challenges are even greater for transgender and nonbinary older adults, who are less likely to be homeowners than their LGB peers.⁵⁷ Housing discrimination, e.g. the denial of rental applications or being turned away at nursing facilities, contributes to difficulty in finding housing, as well as creating a culture of fear for those trying to find a safe place to live. Thirty four percent of LGBTQ+ older adults worry they will have to hide their identity in senior housing,⁵⁸ and 48% of older same-sex couples faced housing discrimination when seeking elder housing.⁵⁹ While nearly 23% of transgender individuals report experiences with housing discrimination.⁶⁰ Older adults with HIV continue to face discrimination in accessing housing as well, including in the context of nursing home admissions.⁶¹

Unfortunately, LGBTQ+ housing discrimination protections have not been enacted and enforced uniformly across the country. Landmark decisions were made under the Biden administration to expand the Fair Housing Act (FHA) to bar discrimination on the basis of sexual orientation and gender identity based on the Supreme Court's decision in *Bostock v. Clayton County*.⁶² However, many states and territories still lack explicit laws to prevent LGBTQ+ housing discrimination, and the current administration has taken steps to undermine *Bostock* and other federal housing protections. This includes the Equal Access Rule (EAR), which is a series of regulations that ensure many U.S. Department of Housing and Urban Development (HUD) funded services are not denied based on

sexual orientation, gender identity, or marital status. Unfortunately, on February 5, 2025, the administration announced it would stop enforcing the EAR, and soon after began the process to officially repeal the rule.⁶³

Beyond undermining discrimination protections, the administration has moved to implement executive orders targeting diversity, equity, and inclusion. This has meant many LGBTQ+ affirming organizations who provide homeless services and support victims of domestic violence, have had their federal grant funding revoked or threatened.⁶⁴ The loss of affirming service providers, which aim to serve everyone whether they are a member of the LGBTQ+ community or not, removes one of the only safety nets available to LGBTQ+ older adults.

Older Americans Act Programs

The Older Americans Act (OAA) is the leading federal statute that addresses the needs of people age 65 and over. It encompasses a host of safety-net services, such as nutrition supports (e.g., home-delivered meals, congregate meal sites), support for caregivers, long-term care ombudsmen, and legal support.⁶⁵ Local Area Agencies on Aging are the primary vehicle for delivering these services to older adults in communities throughout the U.S. and often collaborate with a network of state and local agencies and nonprofits to meet the needs of older adults. These stakeholders are required to target service delivery to marginalized older adults, including those facing social and geographical isolation.

WHAT SERVICE PROVIDERS CAN DO TO BETTER SERVE LGBTQ+ OLDER ADULTS

LGBTQ+ older adults experience social isolation at higher rates than non-LGBTQ+ counterparts due to multiple reasons and therefore may disproportionately rely on public social services as they age.⁶⁶ Lifelong experiences of discrimination can lead to fear, lack of trust, and strains on the relationships of LGBTQ+ older adults.⁶⁷ Some LGBTQ+ older adults may be estranged from their families and similarly lack supportive community resources, a barrier that may be exacerbated in rural areas. As a result, LGBTQ+ older adults are more likely to experience negative mental health outcomes (such as depression) and physical health outcomes (such as increased risk of dementia, heart disease, and stroke), as well as additional functional challenges if aging alone.⁶⁸ Older adults with HIV face many of these same inequities and disparities and lack the social supports needed to age with dignity.

To ensure that LGBTQ+ older adults can fully access the services they deserve, the service delivery must be culturally competent, gender affirming, and welcoming. Targeted outreach to LGBTQ+ older adults is necessary as many may avoid engaging with government agencies and related programs out of fear of discrimination. This outreach can involve connecting with trusted LGBTQ+ advocacy groups, ensuring that the marketing methods are LGBTQ+ friendly (e.g., diversity in imagery, diversity statements, adding SOGI to intake forms), and training direct services staff.

In today's shifting political climate, aging service agencies and service providers may feel pressure to reduce public visibility in

their work with LGBTQ+ elders. However, such withdrawal can be deeply harmful and potentially be perceived as abandonment by older adults already facing heightened fear and isolation. Agencies must find discreet, privacy-conscious ways to maintain and strengthen relationships. This includes consulting legal counsel to assess risks and continuing outreach through trusted channels and materials that do not explicitly identify LGBTQ+ content.

Strategic partnerships are essential. Agencies should collaborate not only with LGBTQ+ organizations, but with cultural, faith-based, legal, and advocacy groups as well to share resources and visibility. Importantly, agencies are encouraged to shift from a hierarchical service model to a collaborative one, empowering LGBTQ+ older adults as partners rather than clients. Hosting focus groups can help co-create solutions, foster community, and identify isolated individuals in need of support. Volunteer roles, such as greeters, teachers, or speakers, can highlight elder strengths and build resilience.

Privacy remains a central concern, especially given LGBTQ+ older adults' experiences with discrimination and violence. Agencies must work with older adults to design service models that protect their identities while still offering meaningful support. Staff, too, may find hope and purpose in these partnerships, even if they participate as volunteers. Ultimately, this moment calls for compassionate, creative, and community-driven approaches to ensure LGBTQ+ older adults remain connected, supported, and empowered.

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[I’m] worried how policy will **impact my ability to find an Assisted Living place**. [I] worry too about the idea of having to completely hide who I am in public, on social media, with police, and VA care.

—ANONYMOUS, LGBTQ+ OLDER ADULT

Nutrition Supports

Inequities in access to nutrition and nutrition supports are also prevalent for LGBTQ+ older adults. LGBTQ+ individuals disproportionately live in poverty, contributing to lack of funds for food, and receive Supplemental Nutrition Assistance Program (SNAP) benefits at twice the rate of non-LGBTQ+ people.⁶⁹ Nutrition related disparities for the broader LGBTQ+ population can arise due to societal factors, such as lack of culturally responsive health care providers and nutrition counseling; elevated stress, including as a result of discrimination and isolation and resulting mental health conditions and coping mechanisms; discrimination in and mistrust of safety net programs; and economic insecurity, which directly impacts the ability to afford food and impacts access to housing, health care, and other protective factors.⁷⁰ Nutrition support programs—such as congregate meal sites, home-delivered meals, SNAP—are essential for LGBTQ+ older adults who cannot otherwise afford groceries, are unable to independently shop for groceries or cook, or who need medically appropriate meals.⁷¹

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The biggest issue I see regarding food insecurity at Edie Windsor SAGE Center, understanding we live in Manhattan, **it’s not an issue of availability, it’s an issue of cost**. People like our members have to decide where to spend their fixed income...medicine, rent, food, clothes, and that means they have to decide what’s a priority, and often times they cut food out. **Any supplemental income and programs like SAGE’s pantries, lunch and dinner service, and Grab-&-Go feeding programs go a long way** to making sure SAGER’s get at least one healthy balanced meal, and hopefully can save some money.

—BOB GURECKI, FOOD SERVICES
COORDINATOR, EDIE WINDSOR
SAGE CENTER

CONCLUSION AND ADDITIONAL RESOURCES

The current administration continues to attack the LGBTQ+ community through its homophobic and transphobic rhetoric, dismantling of federal protections, anti-DEIA initiatives, and pulling critical funds from programs that support health and economic security. These efforts will only widen the existing inequities that LGBTQ+ older adults experience in every aspect of life. Protecting, expanding, and improving the federal programs and strategies discussed in this issue brief are necessary to support LGBTQ+ older adults as they age.

ENDNOTES

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- 22 Brad Sears et al., Williams Institute, [LGBTQ People's Experiences of Workplace Discrimination and Harassment](#) (2024) (noting that a third of LGBTQ employees report leaving a job at some point due to treatment because of their gender identity or sexual orientation).
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